



Interprofessional health teams for integrated care

PAHO



Pan American
Health
Organization



World Health
Organization
Americas Region

Interprofessional health teams for integrated care

Washington, D.C., 2025

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HSS/HRH/2025

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Preface

The chronic shortage of human resources for health is well recognized as one of the main obstacles to delivering effective health services to those who need them most. For millions of people, the shortage of health personnel means a lack of access to essential health services and lifesaving interventions. For health workers, it translates into an excessive workload, stress, fatigue, poor motivation, and decreased ability to ensure patient safety. The lack of health personnel also hampers health systems' ability to respond to public health emergencies.

The COVID-19 pandemic exacerbated a trend that is expected to continue in the years to come, characterized by increasing demands on health workers along with a smaller number of people entering the health workforce. Latin America and the Caribbean are projected to be among the regions with the most substantial net shortages of health personnel by 2030, with a gap of up to 2 million health professionals expected.

Moreover, health workers are unequally distributed among and within countries, with low levels of retention of personnel in rural and underserved areas, leading to pronounced healthcare disparities.

This situation hinders the progressive expansion of health services and the improvement of their quality, especially at the first level of care, compromising health systems' ability to provide health services for all.

Evidence shows that interprofessional teams can strengthen health systems' ability to meet the population's health needs and respond to health emergencies, improving health outcomes by decreasing morbidity and mortality, increasing patient satisfaction, contributing to health worker retention by reducing extra work and increasing job satisfaction, and reducing medical errors and preventable complications.

This publication seeks to support health system authorities, managers, and teams in gathering evidence-based information and practical tools for the organization of interprofessional teams, especially at the first level of care. In doing so, we hope to contribute to strengthening health systems' ability to respond to the population's health needs and to advance the universal health agenda, to make the right to health a reality for all peoples of the Americas.

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Cecilia Acuña from HSS/HR compiled the information and developed the document. Successive versions were shared and reviewed by HSS/HR team members, with all team members making valuable suggestions and additions to the text.

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Department's Life Course Unit, Primary Health Care and Integrated Services Delivery Unit, and Women's, Maternal, Neonatal, and Reproductive Health Unit, all of which enriched this publication and the understanding of the organization of interprofessional teams.

Thanks are extended for the valuable comments made by PAHO's HSS advisors from the subregional and country offices, especially the human resources for health advisor at the Caribbean Subregional Program Coordination, as well as the HSS advisors from the Brazil, Peru, and Uruguay country offices.

Abbreviations and acronyms

EPHF	essential public health function
NCD	noncommunicable disease
PAHO	Pan American Health Organization
PHC	primary health care
WHO	World Health Organization

Executive summary

Evidence shows that interprofessional teams are essential for improving health systems' ability to meet the population's health needs and for responding to health emergencies. Interprofessional collaboration improves health outcomes by decreasing morbidity and mortality; increasing patient satisfaction; contributing to healthcare worker retention by reducing extra work and increasing job satisfaction; and reducing medical errors and preventable complications.

Several Pan American Health Organization (PAHO) and World Health Organization (WHO) resolutions and other technical documents state the need for organizing interprofessional teams to improve health systems' ability to meet the population's health needs and respond to health emergencies. Given this, the purpose of this document is to support PAHO's technical cooperation efforts in developing and implementing national and local planning processes and strategies to organize, strengthen, and incorporate interprofessional teams into integrated health service delivery networks at all levels of care in the context of primary health care, with an emphasis on the first level of care.

The publication introduces a conceptual framework with core elements for consideration by policymakers and health managers in organizing or strengthening interprofessional teams. It also covers practical aspects to support the integration of interprofessional teams into integrated health service delivery networks in real settings at the local level. The core elements include the following:

- Interprofessional teams work best under a clear, previously defined, collaborative, person-centered model of care.
- Interprofessional teams should be trained and organized by considering them through a public

health lens, based on the essential public health functions.

- Interprofessional teams should be trained and organized to implement action plans that are comprehensive, age, stage-of-life, and disability sensitive, and gender responsive, with a focus on human rights.
- Interprofessional teamwork must be founded on six key principles: clear goals (the primary goal being effective care); shared team identity; shared commitment; clear team roles and responsibilities; interdependence between team members; and integration between work practices.
- To ensure a good relationship among the interprofessional team members, four key factors should be fostered: a mutual respect for each other's roles and expertise; collaborative action for working and learning together; a deep understanding of each other's strengths and limitations; and common goals and values.
- To carry out interprofessional practice, six core competencies must be developed and incorporated under the interprofessional learning domains of teamwork; roles and responsibilities; communication; learning and critical reflection; relationship with and recognizing the needs of the patient, the users of the services, and the community; and ethical practice.

In addition, the publication offers some indicators to assess interprofessional teamwork and to support their continuity after incorporation into a health service delivery network. It also lists five outcomes that may be measured as a result of the implementation process.

Finally, the following eight elements are proposed as the main operational entry points for technical cooperation efforts aimed at assisting countries in the process of starting or strengthening the organization

of interprofessional teams, consisting of actions to be carried out at the regional, subregional, and country levels:

1. Promote and advocate for the organization of interprofessional teams through dialogue with policymakers, health and education authorities, and other relevant partners.
2. Support the development and implementation of policies, plans, strategies, regulations, and standards to increase the availability of human resources for health in underserved areas through interprofessional teamwork.
3. Assist countries in the identification of profiles and competencies of health personnel at the first level of care and identify gaps that may be closed by interprofessional teamwork.
4. Develop guidelines and recommendations for organizing, incorporating, and strengthening interprofessional teams, and expanding their roles according to local needs.
5. Promote changes in the curricula for undergraduate, graduate, and postgraduate education to strengthen collaborative practice in health services from the beginning of educational training.
6. Support training strategies for lifelong learning to increase interprofessional teams' capacities to carry out interprofessional practice.
7. Assist health managers in planning processes to integrate interprofessional teams at the first level of care.
8. Promote the establishment of monitoring and evaluation mechanisms for interprofessional teamwork and disseminate evidence and information on lessons learned related to processes, outcomes, and the impact of interprofessional teamwork.

Introduction

Interprofessional teams are essential for improving health systems' ability to meet the population's health needs and for responding to health emergencies. Multiple studies show that interprofessional collaboration improves health outcomes by decreasing morbidity and mortality; increasing patient satisfaction; contributing to healthcare worker retention by reducing extra work and increasing job satisfaction; and reducing medical errors and preventable complications. As such, improved healthcare collaboration has been cited as a key strategy for healthcare reform (1).

According to the Pan American Health Organization (PAHO) publication *Integrated health services networks: Concepts, policy options and road map for implementation in the Americas* (2), a key factor in successful interprofessional collaboration is having sufficient human resources that are competent, committed, and valued by the health service delivery network. This publication highlights the fact that in an integrated health services delivery network (IHSDN), the mix of competencies can be obtained through the employment of different types of professionals working on the same task (multidisciplinary teams), or through the assignment of multiple tasks to a specific individual (multipurpose worker) (2).

Strategic line of action 2 of Document CSP29/10, Strategy on Human Resources for Universal Access to Health and Universal Health Coverage (3), highlights the need to prioritize interprofessional teams at the first level of care through regulation, standard-setting, and public employment opportunities. The strategy indicates that mechanisms should be created to evaluate and adapt the capacities and profiles of teams at the first level of care, to ensure that essential public health functions (EPHFs) are implemented, address the social determinants of health, and promote the development of an intercultural perspective.

In addition, in Document CD56/10, Rev. 1, Plan of Action on Human Resources for Universal Access to Health and Universal Health Coverage 2018-2023, objective 2.2. is to “develop interprofessional teams at the first level of care with combined competencies in comprehensive care and an intercultural and social determinants approach to health” (4).

Furthermore, Document CD60/6, Policy on the Health Workforce 2030: Strengthening Human Resources for Health to Achieve Resilient Health Systems (5), approved by PAHO Member States in September 2023, indicates how the COVID-19 pandemic exposed the chronic shortfall and poor distribution of human resources for health in the Americas. It also exposes the lack of policies; strategic planning processes; and investment in the production, training, and professional development of a fit-for-purpose health workforce, including measures to protect the well-being of health workers. In this context, interprofessional collaboration can provide innovative, system-transforming solutions to ensure the appropriate supply, mix, and distribution of the health workforce (6).

The policy on the health workforce, emphasizes that health workers were key to responding to the COVID-19 pandemic and are also essential for building resilient health systems for the future. It proposes five lines of action, the third being to **strengthen the organization of interprofessional teams and their incorporation into integrated health service delivery networks based on primary health care (PHC)**. It calls for organizing interprofessional teams at the first level of care, with response capacity, the right skills, and an intercultural and social determinants approach to health being recognized as necessary for dealing with the specific circumstances of populations in conditions of vulnerability, aligning interprofessional work with WHO's *Operational Framework for Primary Health Care* (7). In this context, it is important to note

that in real settings health teams consist not only of professionals but also of technicians and community workers, and this is especially true for interprofessional teams at the first level of care.

The health workforce policy document also states that the main tasks to be carried out by interprofessional teams are promoting health and disseminating health information, applying preventive measures, mitigating health risks, conducting epidemiological surveillance, ensuring timely diagnoses through adequate screening, and ensuring access to treatment, rehabilitation, follow-up, and monitoring for each case – all while applying a comprehensive healthcare approach and incorporating traditional medicine.

Therefore, interprofessional teams can be seen as instrumental in realizing the priorities of PAHO and its Member States on accelerating the elimination

of communicable diseases in the Americas (8) and scaling up and accelerating the integration of comprehensive noncommunicable disease (NCD) services (9) and other essential services into PHC to achieve resilient health systems (10).

In addition, it is important to emphasize that the development of interprofessional teams can be leveraged by the implementation of actions and strategies that foster interprofessional education, which occurs when students from different professions are involved in the teaching–learning process, with interactions favoring the understanding of the roles shared by team members, the articulation of knowledge, and collaboration between them. Given the importance of interprofessional education, this topic will be explored further in future PAHO documents and products.

CHAPTER 1

Purpose of this publication

This publication aims to support PAHO's technical cooperation efforts in developing and implementing national and local planning processes and strategies to organize, incorporate, and strengthen interprofessional teams in integrated health service delivery networks at all levels of care in the context of PHC, with an emphasis on the first level of care.

1.1. Objectives

- Identify and analyze the main elements/entry points for leveraging PAHO's technical cooperation efforts in the organization of interprofessional teams.
- Propose a conceptual framework to guide the organization, integration, and strengthening of interprofessional teams at the national, subnational, and local levels.
- Provide guidelines for leveraging PAHO's technical cooperation efforts in the organization of interprofessional teams.

CHAPTER 2

Background

The concept of collaborative work among health personnel working in interprofessional teams is not new. Almost 50 years of research and practice around the world provide evidence to demonstrate that interprofessional education enables effective collaborative practice, which in turn optimizes health services, strengthens health systems, and improves health outcomes (6). Collaborative practice is regarded a prerequisite for high-quality, safe health care for the general population, and important for enabling more effective working practices for professionals and the health system. In both acute and primary care settings, patients report higher levels of satisfaction, better acceptance of care, and improved health outcomes following treatment and rehabilitation by a collaborative team (6). Furthermore, evidence collected during the COVID-19 pandemic shows that collaborative team practices were relevant in organizing the teams who were facing stress in the emergency, fear of contamination, and conflicts (11).

Moreover, interprofessional education and collaborative practice are considered credible strategies that can help mitigate the global health workforce crisis. Evidence shows that collaborative practice has clear benefits for patients and practitioners, and for the health system as well, as shown in Table 1 (12).

Along with the seminal work *Interprofessional teamwork for health and social care* (13),

in which the complex array of elements, factors, and issues that affect how professionals work together is analyzed in depth, an important number of documents have been developed to guide interprofessional education and the organization of interprofessional teams at the country level.

In addition, several resolutions and guidelines refer to collaborative practice and mandate PAHO and WHO to promote interprofessional education and implement interprofessional teams. Moreover, countries such as Brazil, Canada, Chile, Costa Rica, Denmark, India, Japan, Nepal, Oman, Sweden, the United Kingdom, and the United States of America have developed interventions based on interprofessional teams (6, 14).

However, organizing interprofessional teams in real healthcare settings in the Region of the Americas has proved more challenging than expected, and several challenges have been identified to explain this situation. Tackling these challenges requires strong political will, changes in the legal and regulatory framework, and, most importantly, a shift in health course curricula at universities and in continuing education and training at healthcare facilities, to foster working relationships that are less hierarchical, mutual trust, and the sharing of responsibilities for patients' health outcomes (Table 2) (4, 6, 12).

TABLE 1. Benefits of collaborative practice

Can improve/increase	Can reduce
• Access to and coordination of health services • Appropriate use of specialist clinical resources • Health outcomes for people with chronic diseases • Patient care and safety • Clinical outcomes for people with diabetes • Patient and career satisfaction • Acceptance of treatment • Treatment for psychiatric disorders	• Total patient complications • Length of hospital stays • Tension and conflict among caregivers • Staff turnover • Hospital admissions • Clinical error rates • Mortality • Emergency department visits for high-risk patients • Hospitalizations for high-risk patients • Duration of treatment • Cost of care • Incidence of suicide in community mental health settings • Outpatient visits

Sources: World Health Organization. Framework for action on interprofessional education & collaborative practice. Geneva: WHO; 2010. Available from: <https://www.who.int/publications/i/item/framework-for-action-on-interprofessional-education-collaborative-practice>; Guck TP, Potthoff MR, Walters RW, Doll J, Greene MA, DeFreece T. Improved outcomes associated with interprofessional collaborative practice. *Ann Fam Med*. 2019;17:S82–S82. Available from: <https://doi.org/10.1370/afm.2428>; and Reeves S, Lewin S, Espin S, Zwarenstein M. *Interprofessional teamwork for health and social care*. London: Blackwell-Wiley; 2010. Available from: <https://onlinelibrary.wiley.com/doi/book/10.1002/9781444325027>.

TABLE 2. Challenges in the organization of interprofessional teams in real healthcare settings and possible solutions to overcome them

Challenge	Possible solution
Most health teams still work under a disease-centered model of care instead of a people-centered, territory-based model of care.	Given that a disease-centered model of care is predominant in many healthcare settings in the Americas, strong and sustained political will is required to make the shift, along with multiple administrative and organizational changes.
Fragmentation of the legal frameworks for the health, education, labor, and financial sectors, and for regulatory and management practice, limits intersectoral cooperation and hinders the incorporation of interprofessional teams with the competency profiles required by integrated health delivery networks.	Active intersectoral engagement may be fostered to overcome the fragmentation of the legal and regulatory frameworks, and to achieve joint and adequate management practice.
Many universities and health education centers continue to teach and train students around highly hierarchical, disease-centered models of care.	Changes are needed in the curricula to move toward focusing on people-centered care and strategies to strengthen collaborative practice and social integration from the beginning of educational training. Educators should be trained to design and implement interprofessional curricula.
The hierarchical relationship between physicians and other health professionals and health workers hampers collaborative practice.	A shift in health education and in the organization processes at healthcare facilities is needed to achieve less hierarchical, more democratic relationships within health teams.
The expansion of roles of general practitioners, nurses, and other health professionals and technicians has faced challenges for implementation at the country level.	Team members should be trained to share roles and responsibilities based on mutual trust, evidence-based practice and protocols, and joint continuing education.
Significant salary gaps between the members of the health teams as well as the lack of incentives do not facilitate building interprofessional teams.	An increase in salaries and other nonfinancial incentives should be considered for members of the team who expand their roles and responsibilities due to collaborative work in interprofessional teams.
Interprofessional education and collaborative practice can be difficult concepts to explain, understand, and implement.	Collaborative practice goes beyond health workers working together. Collaboration occurs when two or more individuals from different backgrounds with complementary skills interact to create a shared understanding that no one had previously possessed or could have come to on their own.

Sources: Developed by the authors (PAHO Health Systems and Services Department's Human Resources for Health Unit) and based on World Health Organization. Framework for action on interprofessional education & collaborative practice. Geneva: WHO; 2010. Available from: <https://www.who.int/publications/i/item/framework-for-action-on-interprofessional-education-collaborative-practice>; Guck TP, Potthoff MR, Walters RW, Doll J, Greene MA, DeFreece T. Improved outcomes associated with interprofessional collaborative practice. *Ann Fam Med*. 2019;17:S82–S82. Available from: <https://doi.org/10.1370/afm.2428>; and Reeves S, Lewin S, Espin S, Zwarenstein M. *Interprofessional teamwork for health and social care*. London: Blackwell-Wiley; 2010. Available from: <https://onlinelibrary.wiley.com/doi/book/10.1002/9781444325027>.

Overcoming these challenges must be at the center of interventions aimed at organizing, establishing, and strengthening interprofessional teams.

CHAPTER 3

Conceptual framework

3.1. Interprofessional teams work best under a previously defined collaborative and person-, family-, and community-centered model of care

In the context of PAHO's mandates and technical cooperation, interprofessional teams are considered an integral component of person-centered models of care within health systems based on PHC under the principles of universal health, equity, and solidarity, and the right of all people to enjoy the highest attainable standard of health. Human resources for health play a pivotal role in progressively overcoming geographical, economic, sociocultural, organizational, ethnic, and gender barriers so that all communities may have equitable access, without discrimination, to comprehensive health services that are appropriate, timely, effective, and of good quality. In addition, interprofessional collaboration under a person-centered model of care allows for the implementation of a life course approach to health, providing specific care for a targeted population at specific stages of their life, effectively addressing the population's health needs.

From this standpoint, interprofessional teams are crucial for the implementation of person-centered models of care because they provide comprehensive, high-quality care, including by providing a public health perspective, working in a collaborative fashion within integrated healthcare delivery networks and taking into account the needs and diversity of the population under their care.

The collaborative model of care should be previously known and agreed on by all members

of the interprofessional team; its implementation depends on committed and supportive leadership; and it must encompass strong and effective communication mechanisms, collaborative actions across the continuum of care, and ongoing joint staff development and education activities. Collaboration should be promoted among health teams at all levels of care and also with other sectors, such as the social protection sector, in a way that allows the interprofessional team to track down, using a life course perspective, the trajectory of people/patients as they move through the health system and until they are back in the community. This should continue for each episode of care out of the community and/or out of the first level of care or until a person/patient moves permanently out of the community because of either migration or death.

The relevance of the relationship between person-centered care and collaborative interprofessional practice lies in the mutual and bidirectional influence through which the organization of interprofessional teamwork is established within health service delivery networks.

Three distinctive key elements can be noted in this context (15):

1. An expanded perspective of health care is needed to respond to the health needs of persons, families, and communities without reducing them to the dimensions of pathology and physiology, seeking to build health and integrate the promotion, prevention, recovery, and rehabilitation of health throughout a person's life course.

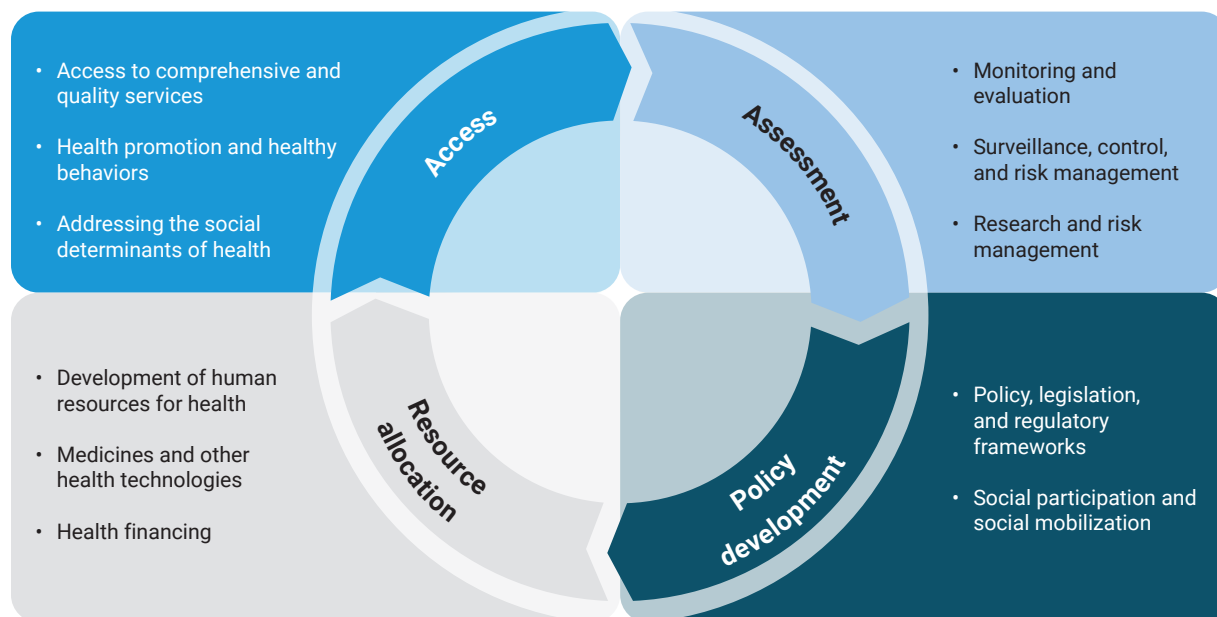
2. People, families, and communities should participate in their own care, acknowledging their uniqueness, moral competency, and self-awareness in the process of participating in their health care, while at the same time reducing inequalities.
3. A collective interprofessional relationship is important between the health team and the user (person/patient), extending collaborative practice to the activities carried out with the user, family, and community, along with the interactions within the interprofessional team, as opposed to individual interactions with patients, families, and communities that lead to isolated practice. In this context, engaging with communities seems to be more about building relationships than individual interactions. Getting involved in dialogue with an entire community triggers reciprocal mechanisms that facilitate back-and-forth communication (15, 16).

3.2. Interprofessional teams and essential public health functions

Public health capacities are key to improving health and well-being on a large scale based on the EPHFs (Figure 1). Interprofessional teams must have a clear understanding of their role in promoting EPHFs at the local level, with emphasis on the following (17):

- **EPHF1:** monitoring and evaluation of health and well-being, equity, social determinants of health, and health system performance and impact;
- **EPHF2:** public health surveillance; control and management of health risks and emergencies;
- **EPHF5:** social participation and social mobilization, inclusion of strategic actors, and transparency;
- **EPHF 6:** development of human resources for health;
- **EPHF7:** ensuring access to and rational use of quality, safety, and effective essential medicines and other health technologies;

FIGURE 1. Essential public health functions



Source: Pan American Health Organization. The essential public health functions in the Americas: a renewal for the 21st century – conceptual framework and description. Washington D.C.: PAHO; 2020. Available from: <https://iris.paho.org/handle/10665.2/53124>.

- **EPHF9:** equitable access to comprehensive and quality health services;
- **EPHF10:** equitable access to interventions that seek to promote health, reduce risk factors, and promote healthy behaviors;
- **EPHF11:** management and promotion of interventions on the social determinants of health.

3.3. Interprofessional teams comprehensive action plans

Interprofessional teams should be able to support and implement at the local level action plans that are comprehensive, age, stage-of-life, and disability sensitive, and gender responsive, while ensuring respect for human rights, with an emphasis on the needs of people in vulnerable situations, promoting social cohesion and supporting interventions to prevent insecurity, violence, discrimination, stigmatization, and marginalization in the community (17).

3.4. Basic principles for organizing interprofessional teams

To succeed in this endeavor, interprofessional teamwork must be founded on the following six key principles (13):

1. Clear goals (the primary goal being effective care);
2. Shared team identity;
3. Shared commitment;
4. Clear team roles and responsibilities;
5. Interdependence between team members;
6. Integration between work practices.

3.5. Key factors for ensuring good working relationships among team members

Good working relationships among interprofessional team members are crucial for ensuring good outcomes of collaborative practice. If relationships are poor among team members,

FIGURE 2. Key factors for ensuring good working relationships among interprofessional team members



Source: Adapted from NHS Improving Quality. Building collaborative teams – a workshop guide for service managers and facilitators. London: NHS England; 2014. Available from: <https://www.england.nhs.uk/improvement-hub/wp-content/uploads/sites/44/2019/01/Building-Collaborative-Teams-workshop-guide-2014-1.pdf>.

acknowledging this at the beginning can help to improve them. If the relationships are already good, the team can decide what will make them even better. The following four factors are key to ensuring that team members have good working relationships (Figure 2) (18):

- Mutual respect for each other’s role and expertise;
- Collaborative action for working and learning together;

- Deep understanding of each other’s strengths and limitations;
- Common goals and values.

3.6. Core competencies to be developed by healthcare workers to adequately engage in interprofessional teamwork

In addition, **six core competencies** for carrying out interprofessional practice must be developed and incorporated into the interprofessional learning domains (Table 3) (4).

TABLE 3. Core competencies for interprofessional teamwork

Core competency	Learning domain
1. Teamwork	Being able to be both a team leader and a team member Knowing the barriers to teamwork
2. Roles and responsibilities	Understanding one’s own roles, responsibilities, and expertise, and that of other types of health workers
3. Communication	Expressing one’s opinions competently to colleagues Listening to team members
4. Learning and critical reflection	Reflecting critically on one’s own relationship within a team Transferring interprofessional learning to the work setting
5. Relationship with and recognizing the needs of the patients, the users of the services, and the community	Working collaboratively in the best interests of the person Engaging with patients, their families, carers, and communities as partners in care management
6. Ethical practice	Understanding the stereotypical views of other health workers held by self and others Acknowledging that each health worker’s views are equally valid and important

Source: Adapted from World Health Organization. Framework for action on interprofessional education & collaborative practice. Geneva: WHO; 2010. Available from: <https://www.who.int/publications/i/item/framework-for-action-on-interprofessional-education-collaborative-practice>.

CHAPTER 4

Practical aspects aimed at implementation of interprofessional teams in health service delivery networks

EPHF6, that is, the development of human resources for health, is important for guiding the organization of interprofessional teams. It highlights that, in addition to traditional competencies such as medicine, epidemiology, laboratory science, community engagement, health education, and environmental public health, new competencies and abilities should be developed by healthcare workers, including in policy analysis, health situation analysis, the concept of global health, communication, monitoring, evaluation, and quality improvement, and a deeper understanding of the social determinants of health and how partnership with sectors other than health can help to address health determinants.

In addition, the provision of services should be carried out within the framework of systems based in PHC and organized in networks, including the participation of all levels of care and the integration of public health services, and in alignment with PAHO's Strategy on Human Resources for Universal Access to Health and Universal Health Coverage (3), which calls for continuity of quality and integrated care along the life course, centered on the needs of people and being sensitive to gender, ethnic, and cultural differences, with an emphasis on the needs of people in vulnerable situations.

Health managers should consider establishing decent working conditions and social protection mechanisms for protecting workers against occupational hazards in the workplace and compensation packages aligned with the expanded responsibilities of interprofessional team members (expanded roles, task-sharing, working hours, etc.). This, along with incentives for professional training and development, and the inclusion of gender perspectives and inclusivity in new hiring models, may be instrumental in ensuring the appropriate geographical distribution of human resources for health according to the health needs of the population and in fostering retention in underserved areas, thereby maximizing the deployment of the country's available resources.

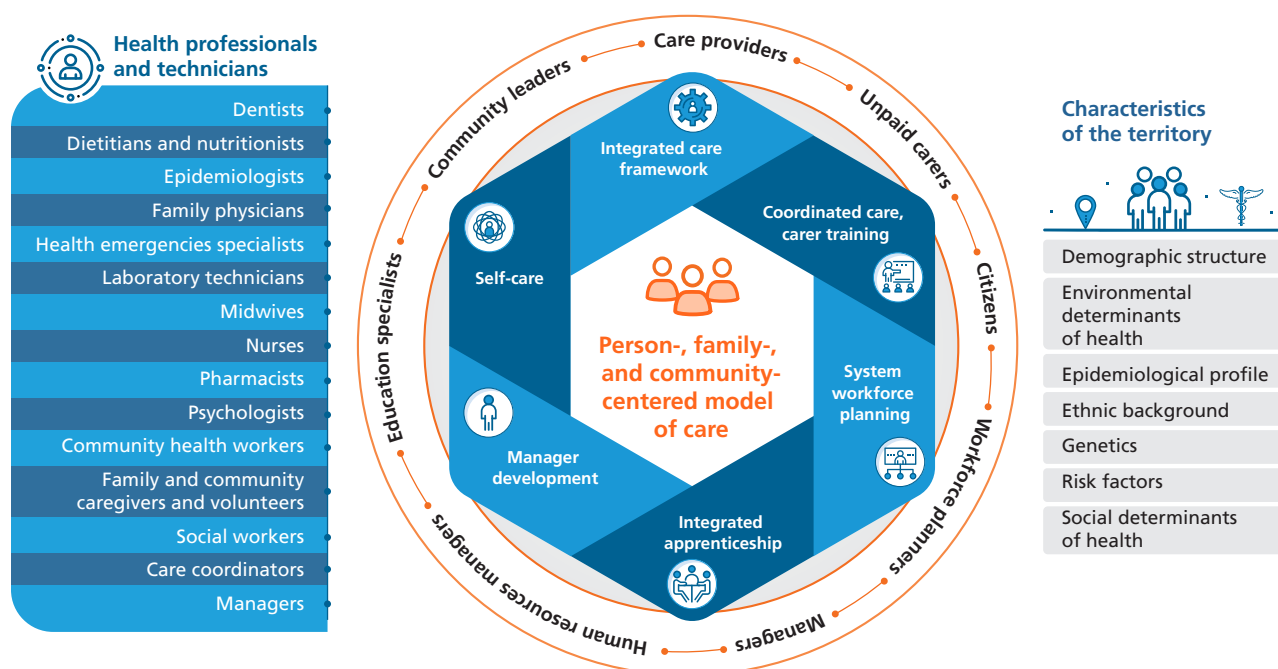
Considering these elements, four practical aspects are proposed for ensuring the successful incorporation of interprofessional teams.

4.1. Key issues to address for the incorporation of interprofessional teams into health service delivery networks

Along with the development of core competencies and technical skills through continued education and training, the successful incorporation of interprofessional teams at the local level will depend on the answers to four key questions:

1. **Where?** The first step is to identify the specific characteristics of the population, including its geographically determined health risks, demographic structure, social and environmental determinants of health, and connectivity, and the distance between the health center and other health facilities that might provide support or serve as a reference if needed.
2. **Who?** Health managers should define the composition of the team (Figure 3) based on the specific characteristics of the population and the territory where team members will work, considering health needs; ethnic backgrounds; cultural preferences and literacy; availability of health professionals and other healthcare providers such as traditional midwives and community health workers; and the capacity of the country to plan and train other healthcare workers. The aim is to maximize the team's capacity through the most suitable skill mix. Given the multisectoral nature of PHC, it is also important to consider how to involve professionals in other sectors, such as the water and sanitation, education, social protection, and environment sectors, as well as those with expertise in engaging people and communities, such as community development specialists or anthropologists (5), under the concept of One Health (19).
3. **What?** The tasks to be carried out and the types of health services to be provided must be clearly defined based on protective and risk factors, the epidemiological profile, and disease burdens. The aim is to improve health results and overcome existing geographical, economic, cultural, or other barriers to accessing care.
4. **How?** The resources for providing care and carrying out the assigned tasks, including health technologies, information systems, and connectivity, must be appropriate and readily available to the interprofessional team. Engaging other sectors could potentially bring in additional resources or redirect existing ones. It is not always necessary to secure new funds, as sometimes reallocating priorities can be sufficient. This flexibility in resource management may be a strength of this approach.

FIGURE 3. Composition of interprofessional teams



Source: Adapted from: Integrated People-Centred Health Services (IPCHS). Integrated care toolkit. Granada: Andalusian School of Public Health; 2020 [cited 10 January 2025]. Available from: <https://www.integratedcare4people.org/toolkits/1552/integrated-care-toolkit/>.

The composition of each interprofessional team will vary according to the population's needs, the characteristics of the territory, the availability of health professionals and technicians, and the capacity of the country to plan and train healthcare workers. The aim is to maximize each team's capacity through its skill mix.

4.2. Types of supporting mechanisms for facilitating the work of interprofessional teams

As mentioned, collaborative practice works effectively when it is organized under person-centered models of care, around the needs of the population being served, and considers the way in which local health care is delivered. Institutional support and the establishment of a specific working culture and specific environmental mechanisms are crucial for the successful incorporation of interprofessional teams at the local level (Table 4 and Figure 4). These mechanisms must promote collaboration, shared responsibility, and integration of practice – as opposed to competition, fragmentation, and isolation – in the delivery of care.

4.3. Dimensions to consider in the provision of care by interprofessional teams

The specific characteristics of the population and the territory not only determine the composition of the interprofessional team but also the type of health services to be provided by the team. Four main dimensions (Table 5) must be considered to identify the services that will be provided and to organize how they are going to be provided:

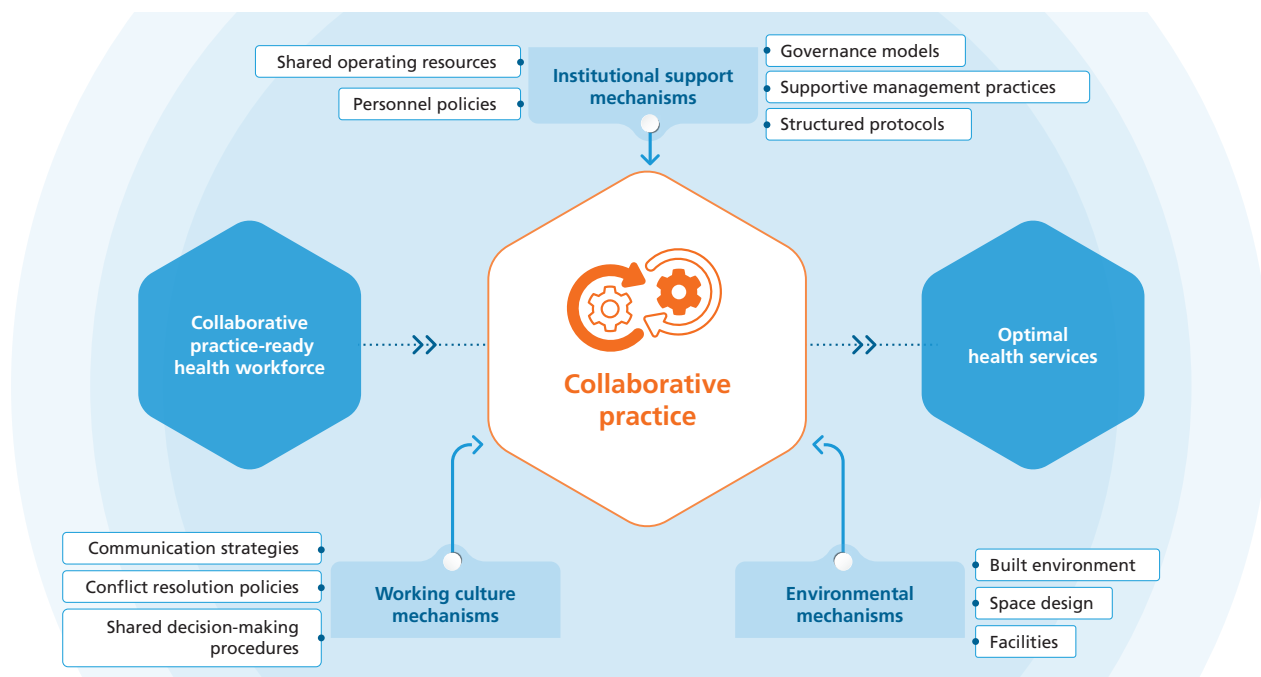
1. Specific groups of the population;
2. Social and environmental determinants of health;
3. Risk factors;
4. Public health priorities.

TABLE 4. Supporting mechanisms for facilitating the establishment of interprofessional teams

Type of support	Examples of mechanisms
Institutional	Clear governance models
	Structured protocols, clinical pathways, and other tools for the delivery of care
	Shared operating procedures
	Personnel policies
	Management practices
Working culture	Communication strategies
	Conflict resolution policies
	Shared decision-making processes
Environmental	Built environment
	Facilities
	Space design

Source: World Health Organization. Framework for action on interprofessional education & collaborative practice. Geneva: WHO; 2010. Available from: <https://www.who.int/publications/i/item/framework-for-action-on-interprofessional-education-collaborative-practice>.

FIGURE 4. Supporting mechanisms for facilitating the establishment of interprofessional teams



Source: Adapted from: World Health Organization. Framework for action on interprofessional education & collaborative practice. Geneva: WHO; 2010. Available from: <https://www.who.int/publications/i/item/framework-for-action-on-interprofessional-education-collaborative-practice>.

TABLE 5. Dimensions to consider in the provision of care by interprofessional teams

Dimension	Characteristics/factors to consider
Specific groups of population	Older adult population
	Small, preterm, and sick newborns, including extremely premature babies and babies with birth defects and disabilities ^a
	Children
	Adolescents
	Reproductive-age women/pregnant women
	Persons with chronic diseases
	People with long-term care needs (persons disabled, bedridden, with limited mobility and/or reduced independence, on palliative care, and with mental health problems) ^{a, b}
	Persons with mental health problems and reduced autonomy
	Caregivers
	Persons living in conditions of vulnerability, Including indigenous peoples, Afro-descendant people, ethnic minorities, migrants, and homeless persons
	LGBTQ population

TABLE 5. Dimensions to consider in the provision of care by interprofessional teams (*continued*)

Dimension	Characteristics/factors to consider
Social and environmental determinants of health	Water availability and quality/sanitation
	Poverty
	Violence
	Isolation and loneliness
	Air pollution
	Soil and water chemical and/or biological pollution (mining activity, use of pesticides, other)
	Seismic/volcanic activity
	Climate change/natural disasters
Risk factors	Tobacco/alcohol consumption
	High blood pressure
	Obesity
	Sedentarism
	Unhealthy diet and malnutrition
	Drug abuse
Public health priorities	NCDs (diabetes, cardiovascular disease, chronic respiratory diseases, cancer)
	Vector-borne diseases
	Zoonotic diseases
	Mother and child health/maternal mortality
	Mental health
	Disability prevention
	Epidemic outbreaks/protracted health emergencies

^a United Nations. Convention on the Rights of Persons with Disabilities. New York: United Nations; 2006 [cited 28 October 2024]. Available from: https://treaties.un.org/pages/viewdetails.aspx?src=treaty&mtdsg_no=iv-15&chapter=4&clang=_en.

^b World Health Organization. Framework for countries to achieve an integrated continuum of long-term care. Geneva: WHO; 2021. Available from: <https://iris.who.int/handle/10665/349911>.

Source: Developed by the authors (PAHO Health Systems and Services Department's Human Resources for Health Unit).

4.4. Types of services to be provided by interprofessional teams

According to the dimensions mentioned above, the interprofessional team may need to organize its daily routine to provide an assortment of intramural and extramural/outreach services (Figure 5). These services may, in turn, be provided in person, virtually

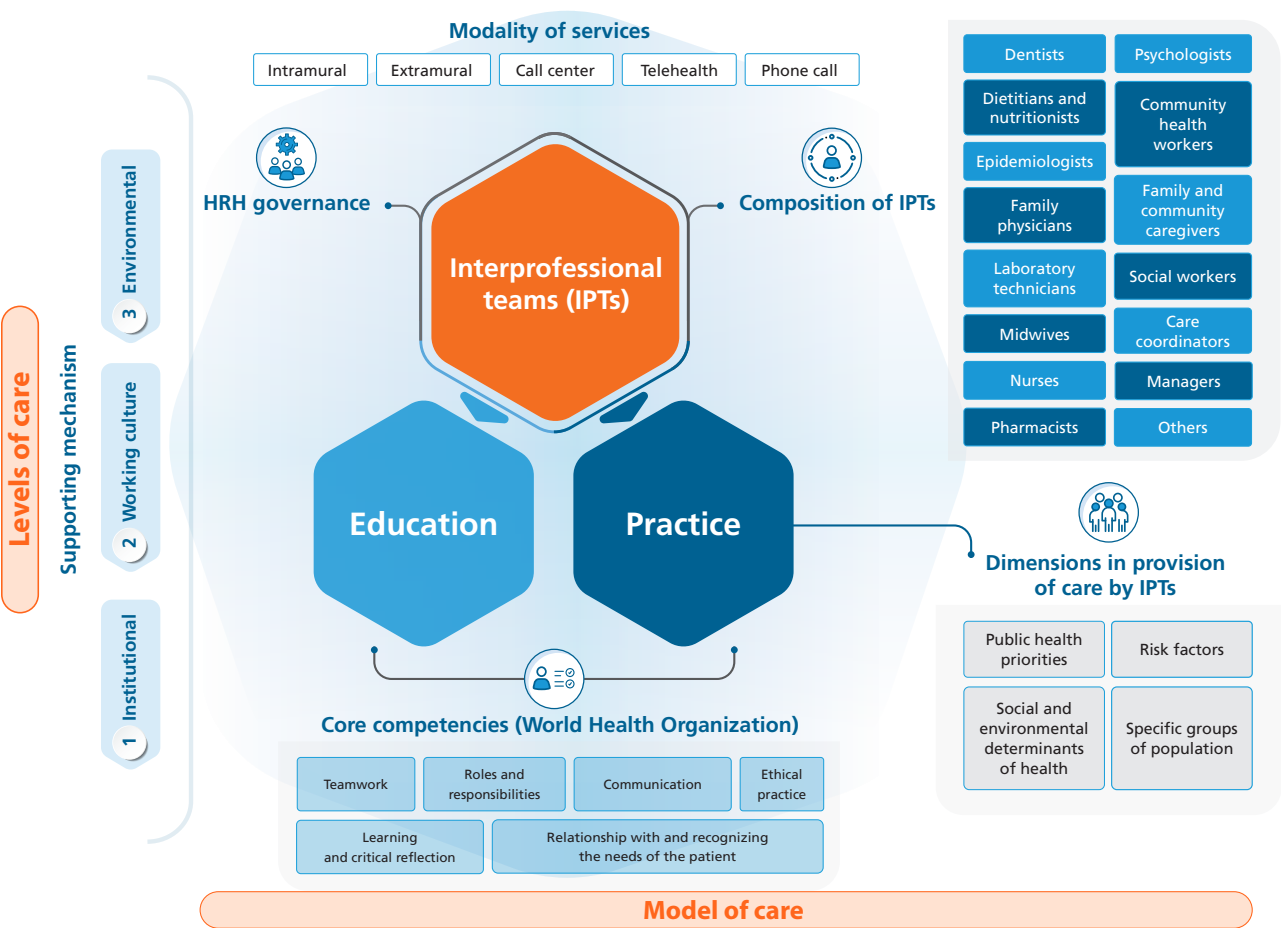
through telehealth, or through call centers/phone calls. The team must be trained to provide specific services that better respond to the population's health needs. In addition, work schedules and task-sharing must be decided on based on the services provided. Table 6 lists the types of services and modalities to be provided by interprofessional teams.

TABLE 6. Type of services to be provided by interprofessional teams

Type of service to be provided by interprofessional team	Modality (intramural/extramural/telehealth/ call center/phone call)
Regular consultation	Intramural/telehealth/call center
Health checkup	Intramural/telehealth
Follow-up of patients with chronic diseases/disabilities	Intramural/extramural/telehealth/phone call
Specialized care for babies with birth defects and disabilities	Intramural/extramural/telehealth/phone call
Antenatal and postnatal care, family planning	Intramural/telehealth
Sexual and reproductive health	Intramural/extramural/telehealth/call center
Oral health	Intramural
Visual and hearing health	Intramural/extramural
Rehabilitation	Intramural/extramural/call center
Long-term care	Intramural/extramural/telehealth
Community care	Extramural
At-home care	Extramural/telehealth/call center/phone call
Psychological first aid/suicide prevention	Intramural/telehealth/call center/phone call
Counseling	Intramural/telehealth/call center/phone call
Management of patients with comorbidities	Intramural/telehealth/call center/phone call
Management of patients with polypharmacy	Intramural/telehealth/call center/phone call
Personal safety education	Intramural/extramural/telehealth
Infection prevention and control	Intramural/extramural/telehealth
Health education and promotion	Intramural/extramural/telehealth
Vaccination and other public health campaigns	Intramural/extramural
Cancer screening	Intramural
Epidemiological surveillance	Intramural/extramural/phone call
Health emergencies preparedness and response	Intramural/extramural

Source: Developed by the authors (PAHO Health Systems and Services Department's Human Resources for Health Unit).

FIGURE 5. Interprofessional team conceptual framework



Note: HRH: human resources for health; IPT: interprofessional team.

Source: Developed by the authors (PAHO Health Systems and Services Department's Human Resources for Health Unit).

CHAPTER 5

Assessing interprofessional teamwork

EPHF6 highlights that an important component of the implementation of interprofessional teams is the introduction of mechanisms to evaluate and adapt capacities and profiles to ensure that all EPHFs are implemented, social determinants of health are addressed, and interculturalism is fostered, especially at the first level of care.

Assessing the results of interprofessional teamwork activities and initiatives is essential for supporting their continuity after incorporation. Such an assessment can help to (13):

- Develop an understanding of the nature of teamwork, including when, how, and why it works, as well as when, how, and why it does not work;

- Identify situations where one form of teamwork is more (or less) effective than others;
- Provide accountability for the health and social care resources invested in organizing teams;
- Identify strengths and weaknesses, to improve the effectiveness and efficiency of the interprofessional team;
- Provide generalizable evidence to support advancing knowledge in the field.

Reeves et al. (13) identify four areas of assessment that can be used to gauge the success of interprofessional team interventions (Table 7).

Furthermore, five immediate outcomes may be measured following the implementation of interprofessional teams (Table 8).

TABLE 7. Areas for the assessment of interprofessional teams

Area of assessment	Considerations
Institutional inputs	Training experiences, new staff added to the team, changes in the legal/regulatory framework, institutional supporting mechanisms in place
Processes	Changes in work processes, routines, task distribution and sharing, and interactions among team members
Outcomes	The extent to which the intervention has: • Changed, increased, or improved collaboration; • Made communication more informative and less interruptive; • Resulted in more appropriate and inclusive consultation and decision-making; • Resulted in increased people's/patients' satisfaction; • Resulted in a reduction in patient complications, avoidable hospital admissions, and visits to the emergency room, and an increase in patients' adherence to treatment

TABLE 7. Areas for the assessment of interprofessional teams (*continued*)

Area of assessment	Considerations
Impact	Changes in people's/patients' health status, social integration, and probability of remaining in a home environment Changes in healthcare workers' job satisfaction, work stress, absenteeism, and turnover

Source: Developed by the authors based on Reeves S, Lewin S, Espin S, Zwarenstein M. Interprofessional teamwork for health and social care. London: Blackwell-Wiley; 2010. Available from: <https://onlinelibrary.wiley.com/doi/book/10.1002/9781444325027>.

TABLE 8. Immediate outcomes

Immediate outcomes following implementation of interprofessional teams	
1	A greater sense of shared values, vision, and purpose
2	A plan on how to work together more effectively
3	Improved relationships between team members
4	A better understanding of each other's roles and challenges
5	An action plan to work on together

Source: NHS Improving Quality. Building collaborative teams – a workshop guide for service managers and facilitators. London: NHS England; 2014. Available from: <https://www.england.nhs.uk/improvement-hub/wp-content/uploads/sites/44/2019/01/Building-Collaborative-Teams-workshop-guide-2014-1.pdf>.

CHAPTER 6

Elements for leveraging PAHO’s technical cooperation efforts in the organization of interprofessional teams

Based on the concepts and information shared, and considering PAHO’s Strategic Plan 2020–2025, which has the health workforce as one of its priorities, and EPHF6 on the development of human resources for health, eight elements are proposed as the main

operational entry points for starting or strengthening the organization of interprofessional teams at the country and local levels, with the aim of allowing these teams to develop the skills needed to carry out specific tasks, as described in Table 9.

TABLE 9. Skills and tasks to be developed by interprofessional teams

Skills to be developed by interprofessional teams	Tasks to be carried out by interprofessional teams
• Response capacity • Appropriate and up-to-date technical skills • An intercultural, social, and environmental determinants approach to health • A public health lens • A gender, ethnicity, and human rights focus • A participatory approach focused on people, families, and communities • An intercultural perspective	• Promoting health and disseminating health information • Applying preventive measures • Mitigating health risks • Conducting epidemiological surveillance and identifying public health issues in the community • Ensuring timely diagnoses through adequate screening • Encouraging self-care and ensuring access to treatment, rehabilitation, follow-up, and monitoring for each case • Applying a comprehensive healthcare approach and incorporating traditional medicine

Source: Based on Pan American Health Organization. Policy on the Health Workforce 2030: Strengthening Human Resources for Health to Achieve Resilient Health Systems [Document CD60/6]. 60th Directing Council, 75th Session of the Regional Committee of WHO for the Americas: 25–29 September 2023. Washington, D.C.: PAHO; 2023. Available from: <https://www.paho.org/en/documents/cd606-policy-health-workforce-2030-strengthening-human-resources-health-achieve-resilient>.

The eight entry points displayed in Table 10 consist of actions to be carried out at the regional, subregional, and country levels as part of PAHO's technical cooperation.

TABLE 10. Eight elements/entry points for leveraging PAHO's technical cooperation efforts in the organization of interprofessional teams

Elements/entry points for leveraging PAHO's technical cooperation efforts in the organization of interprofessional teams	
1.	Promote and advocate for the organization of interprofessional teams actively engaging in dialogue with policymakers, health and education authorities, and other relevant partners and players, using the available evidence.
2.	Support the development and implementation of policies, plans, strategies, regulations, and standards to increase the development, availability, distribution, attraction, and retention of human resources for health in vulnerable and remote areas through interprofessional teamwork.
3.	Assist countries in the identification of professions, profiles, and competencies of health personnel working at the first level of care, and identify gaps related to current and future needs that may be closed through interprofessional teamwork.
4.	Develop guidelines and establish recommendations on organizing, incorporating, and strengthening interprofessional teams, and to define, strengthen, and expand the role of general practitioners, family physicians, nurses, midwives, pharmacists, social workers, psychologists, community health workers, dentists, epidemiologists, and other workers from different professions according to local needs and the availability of health personnel.
5.	Contribute to and promote changes in curricula to transform undergraduate, graduate, and postgraduate education, by focusing on people-centered care and strategies to strengthen collaborative practice in health services from the beginning of educational training.
6.	Support training strategies for lifelong learning to increase interprofessional teams' knowledge of and capacities in people-centered models of care, the essential public health functions, the basic principles of organizing interprofessional teams, and the core competencies needed to carry out interprofessional practice.
7.	Support the implementation of interprofessional teams at the first level of care, assisting health managers in the planning process by identifying key issues for the integration of teams, establishing supporting mechanisms to facilitate their work, identifying the services to be provided, and organizing how they are going to be provided.
8.	Promote the establishment of monitoring and evaluation mechanisms for interprofessional teamwork, and disseminate evidence and information from lessons learned related to processes, outcomes, and the impact of interprofessional teamwork to advance the universal health agenda and improve health outcomes in the Americas.

Source: Developed by the authors (PAHO Health Systems and Services Department's Human Resources for Health Unit).

6.1 The way forward

Interprofessional collaboration not only provides innovative, system-transforming solutions to ensure the appropriate supply, mix, and distribution of the health workforce, but is also instrumental in realizing the priorities of PAHO and its Member States on accelerating the elimination of communicable diseases, scaling up and accelerating the integration of comprehensive NCD services into PHC, reducing maternal mortality, and strengthening preparedness and response

to health emergencies to achieve resilient health systems. To succeed, interprofessional teams must implement a territory-based planning approach and build stronger linkages between health services and the community, in line with the Strategy on Human Resources for Universal Access to Health and Universal Health Coverage (3). To achieve these goals, the organization of interprofessional teams should be an organization-wide effort of technical cooperation.

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Glossary

Collaborative practice

Involves multiple health workers from different professional backgrounds working together with patients, families, carers, and communities to deliver the highest quality of care across settings.

Disciplines

Broad academic fields studied at an academic institution. The key difference between a discipline and a profession is that a discipline is a branch of knowledge learned at the college or university level, whereas a profession refers to any type of work that needs special training or a particular skill to perform.

Interdisciplinary teamwork

Carried out by individuals from different disciplines.

Interprofessional collaboration

Involves different professions meeting regularly to negotiate and agree on how to solve complex care problems or provide services. It differs from interprofessional teamwork in that team members do not share a team identity and work together in a less integrated and interdependent way.

Interprofessional education

Involves members (or students) of two or more professions engaging in learning with, from, and about each other to improve collaboration and the delivery of care.

Interprofessional teamwork

Involves different professions sharing a team identity and working together in an integrated and interdependent way to solve complex care problems and provide services.

Intraprofessional activity

An activity carried out by individuals from the same profession.

Multiprofessional teamwork

Characterized by working in a team made up of members from different professions who work in parallel rather than interactively.

Profession

A type of job that requires advanced education or training. An individual needs special training or a set of specific skills to be engaged in a profession. Discipline and profession are interrelated terms. In a discipline, students gain knowledge, while in a profession the individuals (i.e., the professionals) apply this knowledge in practice.

Transdisciplinary practice

Involves an individual from one discipline taking on a function or task from another discipline. Although this function or task is outside that individual's usual scope of practice, it is assumed that they have the necessary knowledge to carry it out.

Transprofessional practice

Involves an individual from one professional group taking on the functions or tasks of another professional group. Although these professional functions or tasks are outside that individual's usual scope of practice, it is assumed that they have the necessary knowledge to carry them out.

Unidisciplinary activity

An activity carried out by only one discipline.

Uniprofessional activity

An activity carried out by only one profession.

Sources: Journal of Interprofessional Care. Terminology. Abingdon: Taylor & Francis Group; [date unknown] [cited 30 October 2023]. Available from: <https://files.taylorandfrancis.com/ijic-terminology.pdf>; World Health Organization. Framework for action on interprofessional education & collaborative practice. Geneva: WHO; 2010. Available from: <https://www.who.int/publications/i/item/framework-for-action-on-interprofessional-education-collaborative-practice>; and Kumari G. What is the difference between discipline and profession. DifferenceBetween.com; 2022. Available from: <https://www.differencebetween.com/what-is-the-difference-between-discipline-and-profession/>.

Framework for action

The framework for action outlined in this publication incorporates concepts, definitions, topics, and mandates from the following PAHO and WHO resolutions, policy documents, and guidelines:

- PAHO: Document CD60/6 – Policy on the Health Workforce 2030: Strengthening Human Resources for Health to Achieve Resilient Health Systems (2023).
- PAHO: Resolution CD52.R13 – Human resources for health: increasing access to qualified health workers in primary health care-based health systems (2013).
- PAHO: Resolution CD53.R14 – Strategy for Universal Access to Health and Universal Health Coverage (2014).
- PAHO: Resolution CSP29.10 – Strategy on Human Resources for Universal Access to Health and Universal Health Coverage (2017).
- WHO: Framework for action on interprofessional education & collaborative practice (2010).
- WHO and United Nations Children’s Fund: Operational framework for primary health care: transforming vision into action (2020).
- WHO: Resolution WHA69.19 – Global strategy on human resources for health: workforce 2030 (2016).
- PAHO: Resolution CD47.R9 – Regional Strategy and Plan of Action on an Integrated Approach to the Prevention and Control of Chronic Diseases, Including Diet and Physical Activity (2006).
- PAHO: Document CD52/7, Rev. 1 – Plan of Action for the Prevention and Control of Noncommunicable Diseases in the Americas 2013–2019 (2013).
- The Strategic Plan of the Pan American Health Organization 2020–2025: Equity at the Heart of Health, with four cross-cutting themes that are central to this Plan’s approach to addressing the determinants of health: equity, gender, ethnicity, and human rights.

Evidence shows that interprofessional teams are essential for improving health systems' ability to meet the population's health needs and respond to health emergencies.

Interprofessional collaboration improves health outcomes by decreasing morbidity and mortality; increasing patient satisfaction; contributing to healthcare worker retention by reducing extra work and increasing job satisfaction; and reducing medical errors and preventable complications. However, organizing interprofessional teams in real healthcare settings in the Region of the Americas has proved more challenging than expected.

This publication introduces a conceptual framework with core elements for consideration by policymakers and health managers in organizing and strengthening interprofessional teams. In addition, it provides practical guidance to support and facilitate the incorporation of interprofessional teams into health service delivery networks in real settings at the local level.

It also suggests how to assess interprofessional teamwork and support its continuity after initial incorporation, and lists five outcomes that may be measured as a result of the implementation process.

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